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RESEARCH

Patients' Experiences of Triage and Waiting in Emergency Departments: A Cross-sectional Survey

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ABSTRACT

Objective: Patient experience is a core aspect of care quality. Patient experiences of emergency departments (EDs) have been examined, but there has been less focus on ED triage and waiting areas, often patients' initial point of contact with a health service. This study aimed to understand patients' experiences of ED triage and waiting room. **Methods:** A cross-sectional survey was conducted across five Australian EDs to understand experiences of patients 14 years and over before, during and after triage. Quantitative data were analysed descriptively and explored for correlations. Qualitative responses for suggested improvements were analysed using a conventional content analysis approach. **Results:** A total of 225 respondents completed the survey and reported they were generally satisfied with their triage and waiting room experience. Key areas for improvement were identified. Wait time information was reported to be provided to 24.3% of patients, and pain relief to 52.7% of those who desired it. Following triage respondents reported spending a median time of 60 minutes in the waiting room. Satisfaction with waiting was negatively correlated with increasing wait time ($p < 0.001$). Suggestions for improvements reflected a need for better staffing, facilities, and processes to support ED efficiencies and care needs while waiting. **Conclusions:** Wait time was a key influence on reported patient experience. Recommended measures to support the provision of information, comfort and person-centred care while waiting could help to improve this experience.

Keywords: Patient experience, Patient satisfaction, Patient-centered care, Emergency room visits, Triage, Surveys and questionnaires

1. Introduction

Patient experience is an important contributor to healthcare quality. It has reported associations with clinical outcomes and patient safety,[1,2] and can influence patient behaviour, patient loyalty, and organisational financial performance.[3] Additionally, patient experience is integral to person-centred care, which has become a key standard for healthcare globally.[4–6] Understanding the benefits to patients and organisations, health services have adopted patient-reported experience measures to assess aspects of care, including clinician-patient interactions, perceived benefits and interactions with health services and systems.[7] The patient's experience involves both independent and collective occurrences across the continuum of care, and is often tied to their expectations for care.[8] Elements of patient experience can include:

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feeling safe, smooth hospital transitions, treatment that meets patient expectations, effective and timely pain management, clear information and communication, being listened to, staff responsiveness, consistency of care quality, clean and comfortable environment, sleep/rest quality and food quality.[9] Measuring patient experience elements can support health services and departments identify areas for improvement, and drive and evaluate interventions to address them.[10]

Patients' experiences of attending emergency departments (EDs) have been examined worldwide in recent decades. A systematic review of 107 articles published up until mid-2015 identified key themes for patient experience of ED as being: staff-patient communication, ED wait times, staff empathy and compassion, patient demographic factors, ED environment, patient expectations and pain/pain control.[11] Graham et al[12] conducted a meta-synthesis of qualitative research published to 2018 and offered a conceptual framework for patient experience in the ED that incorporated determinates (personal, technical, cultural, physical and environmental), patient needs (communication, emotional, care, waiting, physical/environmental) and clinical practice recommendations. More recently, patient experience surveys and findings from interviews about ED have been reported, with similar themes, further emphasis on clinician-patient relationships, the need for information, timely care, and having a companion in the department.[13–15]

Whilst patient experience in the ED has been explored, how patients experience ED triage and waiting room specifically has been less commonly identified, and usually formed part of an evaluation of triage interventions.[16] An integrative review identified that wait times, instigation of assessment and treatment while waiting, information provision and interactions with triage staff most commonly impacted patient experience, and that patients' expectations for these varied.[16] Authors suggested a person-centred approach to ED triage that is informed by an understanding of individuals' needs for waiting, information, comfort and communication with staff. That is, providing care that is meaningful to the patient. Research to understand these needs specifically in the ED triage area is lacking. The purpose of this study was to understand patients' experiences of ED triage and waiting rooms, as part of a larger project to develop a person-centred framework for ED triage.

2. Methods

2.1. Design

A cross-sectional survey was undertaken to describe patients' experiences of triage and the waiting room, guided by Grey's five-step process[17] of: 1) design and preliminary planning, 2) pre-testing, 3) final survey design and planning, 4) data collection, 5) data coding, analysis and reporting. A reference group of patient and nurse representatives who had recent experience of ED triage contributed to the study design and development of survey questions and participant information documents, testing of the tool, and planning recruitment methods. Survey results were presented to the reference group for their feedback and interpretation. The study is reported following the consensus-based checklist for reporting of survey studies (CROSS).[18]

2.2. Survey tool

A review of scholarly articles[16] and grey literature identified several survey tools for measuring patient experience of hospitals broadly,[19–21] or the ED generally,[22–27] but none that examined experience of triage areas and elements explicitly. Thus, a 35-question survey tool was purposively developed and tested for this study (Appendix I). Initial questions were based on the literature review[16] and triage questions from relevant ED experience surveys: the Consumer Quality Index for the Accident and Emergency department (CQI A&E, Netherlands),[28] National Health Service Accident & Emergency Department Questionnaire (United Kingdom),[29] and Queensland Health Emergency Department Patient Experience Survey (QLD EDPES, Australia).[22] Four sections collected information about 1) respondent characteristics and experiences 2) prior, 3) during and 4) after triage (before being taken into the main department). Patients were asked to indicate yes/no/somewhat or select a score on a rating scale for questions about elements of their experience, such as "was the triage nurse polite to you?" and "on a scale of 0 to 10, how much pain did you have when you arrived at the ED?" respectively. Two questions asked patients to document their wait times to triage and in waiting area. A final open-ended question invited patients to comment on their experience and provide suggestions for improvements. Three experience sections were tested by a panel of seven health consumers for clarity, content validity and apparent internal consistency, and achieved agreement based on a priori criteria[30] (Appendix II).

2.3. Setting and sample

The survey was conducted at five hospital EDs in Western Australia that represented a range of settings: a large tertiary centre, a metropolitan general hospital, a rural regional hospital, a small rural hospital and a remote regional hospital. A similar model of care was utilised at all hospitals whereby patients who presented to the ED were seen and assessed by a triage nurse using the Australasian Triage Scale,[31] registered by clerical staff and then either instructed to sit in a waiting room or taken directly into the main department for medical assessment. All hospitals had a 'fast-track' or 'ambulatory' care triage stream specifically for non-complex presentations that could be easily treated and discharged. Some EDs had additional features, such as processes, facilities and staff to commence investigations and treatment in the waiting room, displayed informational posters and videos about the ED, provided volunteers in the waiting room and food vending machines were available.

All patients aged 14 years and older who were able to understand the survey questions were eligible to participate. Patients were excluded if they were too unwell to participate, as determined through liaison with the nurse coordinator/lead, or unable to recall their triage interaction and waiting experience, such as those who were unconscious on arrival. An interpreter service was available for non-English speaking patients.

2.4. Data collection

Surveys were administered in person for five weeks between July and September 2024. One researcher (CJ) recruited eligible patients for six hours pers day, four days per site. At the time of data collection, average ED presentations at the sites ranged from 21-312 patients per day. It is estimated there were 353 eligible patients across 120 hours of data collection (using average presentations per hour, and exclusions 50% based on state data[32]). Eligible patients in the main ED (following triage) were approached and invited to complete the survey. Some eligible patients may have been missed due to opportunity and timing constraints, such as being in the waiting room, undergoing treatment or care processes, transferred to another location, or discharged prior to being approached. Additionally, patients may have left the ED prior to receiving care. In Australia in 2023-4 this rate was reported as 3.9%.[32] Patients completed the paper survey independently or with assistance from the researcher or family members if needed, for example if the patient was unable to use their hand to write or preferred that the questions were verbalised. Patients only needed to answer questions relevant to their experience.

2.5. Data analysis

Survey responses were manually entered into Qualtrics software (Qualtrics, Provo, UT) for data generation, then exported into Excel spreadsheets and uploaded to IBM SPSS Statistics for Windows (version 29) for analysis. Frequencies, percentages, means, medians, modes and ranges were used to describe quantitative data. Key indicators of patient experience identified from literature; satisfaction, care expectation, pain relief provision and wait time; were cross tabulated against respondent characteristics to identify differences. Kendall's tau b was run to determine the relationship between wait time and satisfaction with wait experience, with significance set at $p = 0.01$ (2-tailed). Missing data were excluding from analyses, and the number of missing values and valid percentages reported.

Qualitative responses from the open-ended question were analysed for codes, categories and sub-categories using a conventional content analysis approach.[33] Codes and categories were created by one researcher (CJ) and peer-reviewed by two researchers (FG, GL). Categories and sub-categories with exemplar comments were presented to the project reference group to check interpretation of results.[17]

2.6. Ethical considerations

Ethical approval was obtained from the South Metropolitan Health Service Human Research Ethics Committee (HREC) (RGS0000006844) and reciprocal approval from Curtin University HREC (HRE2024-0326). Participants were provided an information sheet and consent was requested verbally and implied by completion of the survey. Young people aged 14–18 years of age were assessed by the researcher for maturity and capacity to understand the research by asking five questions about the study once they had read the information sheet and if they provided appropriate answers (Appendix III)[34] Additionally, verbal consent was gained from a parent or guardian. Of the young people assessed ($n = 11$), none were deemed not mature or capable to participate. To minimise the potential for Hawthorne effect[35] and selection bias, ED staff were not informed of the specific details of the research nor were they shown the survey. They were only asked to identify patients who could be approached, i.e. not too unwell, and indicate if infection precautions were required. The researcher did not recruit participants or read survey questions aloud (if requested) in front of ED staff; to ensure patients felt comfortable answering questions about their care. Surveys did not collect identifying information about the participants and all study documents and data were stored safely and securely.

3. Results

3.1. Respondent characteristics

There was a total of 225 surveys, with 99 being partially completed. None were excluded. Fewer than 20 patients declined to participate (not formally recorded). Respondents represented a range of ages groups (12.4% 14–24 years, 28.9% 25–44 years, 26.2% 45–64 years, 32.4% 65 years and over) and diversity (11.1% culturally and linguistically diverse, 13.4% with a disability, 26.4% with a chronic condition, 16.2% with a mental health condition, 2.8% sexually diverse) and a balance of genders (52.7% female, 46.8% male). The majority spoke English as a primary language (92.8%). No respondents required an interpreter. A full list of respondent characteristics is shown in Table 1.

Table 1. Respondent characteristics.

Respondent characteristics (n = 225)	n (valid %)	Missing
Site		0
Hospital A	60 (26.7%)	
Hospital B	58 (25.8%)	
Hospital C	45 (20.0%)	
Hospital D	32 (14.2%)	
Hospital E	30 (13.3%)	
Age group		0
14–24 years	28 (12.4)	
25–44 years	65 (28.9)	
45–64 years	59 (26.2)	
65 years and over	73 (32.4)	
Gender		3
Female	117 (52.7)	
Male	104 (46.8)	
Prefer not to say	1 (0.5)	
Primary language		3
English	206 (92.8)	
Other than English	16 (7.2)	
Country of birth		3
English primary language	187 (84.2)	
English not the primary language	35 (15.8)	
*Identify as**		9
Culturally and linguistically diverse	24 (11.1)	
Person with a disability	29 (13.4)	
Person with a chronic condition	57 (26.4)	
Person with a mental health condition	35 (16.2)	
LGBTIQA + (gender diverse)	6 (2.8)	
None of these	109 (50.5)	
Prefer not to say	2 (0.9)	

Note. Valid % excludes missing values. Hospitals: A=large tertiary centre, B=metropolitan general hospital, C = rural regional hospital, D=small rural hospital and E=remote regional hospital.
*Respondents could select multiple answers.

3.2. Prior to triage

On arrival to the ED, respondents reported a median pain score of 7/10 (IQR 5) and median anxiety level of 7/10 (IQR 3). Over half perceived their condition to be serious or very serious (54.1%), and one-third indicated they were referred by a health professional (38.3%). Most (76.9%) had prior knowledge of triage. Further detail for patient experience prior to, during and after triage are displayed in Table 2.

3.3. During triage

Patient-reported time to triage ranged from 0 to 60 minutes (median 5 minutes, IQR 13 minutes). Satisfaction with the triage nurse was rated very high in most cases (median score 9/10), with majority reporting that the nurse was polite (96.1%), took them seriously (93.2%), listened carefully (95.2%), had enough time for them (94.2%), and asked about pain (92.7%), and that they had confidence in the triage nurse (89.8%). Only about one quarter reported they had been advised of an expected wait time (24.3%) or the reason for their wait (29.0%). Of note, young people aged 14–24 years less often received the care they expected at triage and reported less satisfaction than patients from other age groups (66.7% compared with 87.3–92.7%, and 75% compared with 84.9-93.8% respectively) (Table 3).

3.4. After triage

After triage, respondents reported they had been required to wait between 0 to 660 minutes (median 60 minutes, IQR 197.5 minutes) before being taken into the main department. The median reported wait time was higher for metropolitan EDs (150 minutes) compared with rural EDs (20 minutes). Few reported being updated about their expected wait time (19.7%). Whilst most indicated that staff were available to assist if they needed (81.7%), only half of respondents who desired pain relief reported they were provided it (52.7%). Most indicated they felt safe in the waiting area (84.1%) and around half noted the waiting area to be comfortable (51.2%). Median satisfaction with the waiting area was 8/10 (IQR 3).

Comparison of patient experience indicators with respondent characteristics identified some key

Table 2. Patient experience.

Responses (n = 225)	Answered yes (valid %)	Missing	Median (IQR)	Mode	Range
Prior to triage					
Referred by a health professional	85 (38.3%)	3			
Perceived seriousness		5			
-Not serious	11 (5.0%)				
-Somewhat serious	90 (40.9%)				
-Serious	80 (36.4%)				
-Very serious	39 (17.7%)				
Prior knowledge of triage	170 (76.9%)	4			
Level of anxiety (0–10)		3	7 (3)	8	0–10
Level of pain (0–10)		2	7 (5)	8	0–10
During triage					
Triage nurse introduced themselves	151 (71.6%)	14			
Triage nurse was polite	199 (96.1%)	18			
Triage nurse took me seriously	193 (93.2%)	18			
Triage nurse listened carefully	197 (95.2%)	18			
Triage nurse had enough time for me	194 (94.2%)	19			
Triage nurse asked about pain	191 (92.7%)	19			
Confidence in the triage nurse	185 (89.8%)	19			
Told about wait time	51 (24.3%)	15			
Told reason for wait	56 (29.0%)	32			
Got the care expected at triage	182 (87.5%)	17			
Reported time to triage (minutes)		16	5 (13)	5	0–60
Satisfaction with triage (0–10)		17	9 (12)	10	0–10
After triage (waiting area)					
Wait area		7			
-Waiting room	166 (73.8%)				
-Ambulance ramp	6 (2.7%)				
-Went straight in	45 (20%)				
-Other area	1 (0.4%)				
Accompanied by family/friend	116 (54%)	10			
Pain relief		20			
-Required	129 (62.9%)				
-Given if required	68 (52.7%)				
Staff available to assist	147 (81.7%)	45			
More serious patients seen first	55 (26.6%)	18			
Updated about wait time	37 (19.7%)	37			
Felt worried staff had forgotten me*	31 (15.7%)	27			
Wait area was comfortable	106 (51.2%)	18			
Felt safe in the waiting area	180 (84.1%)	11			
Reported wait time (minutes)		17	60 (197.5)	120	0–660
-Metropolitan (n = 108)		10	150 (189.4)	120	0–660
-Rural (n = 100)		7	20 (55)	5	0–300
Satisfaction with wait (0–10)		7	8 (3)	10	0–10

Note. Valid % excludes missing values. *denotes a negatively worded question.

differences (Table 3). Respondents aged over 65 years (34.1%) and those with a chronic condition (43.2%) reported they were less frequently provided pain relief, when needed, than their counterparts (45.5–82.4% and 59.3% respectively). Respondents aged 14–24 years (161.5 minutes), females (140 minutes) and those born outside of Australia (138.6 minutes) reported longer wait times than their counterparts (75–112.5, 77.9 and 93.1 minutes). Females (59%) and those born outside of Australia (58.8%) were also less satisfied with their wait than males (76.9%) and respondents born in Australia (72.3%). Linear regression showed a moderate negative relationship between wait time and satisfaction with wait (Tb = –0.468, $p < 0.001$) (Fig. 1). The model explained approximately $R^2 = 0.289$ (or 29%) of the variance in satisfaction that can be explained by wait time.

3.5. Suggested improvements

A total 273 comments were provided by 177 respondents. Many (n = 90) commented that their experience as a patient was good or acceptable, and expressed appreciation for the care and attention they received under the circumstances. Eighty-three unique codes were generated and grouped into 19 sub-categories and four key categories that identified

Table 3. Respondent characteristics and experience.

Respondent characteristics	Got care expected at triage Answered yes (valid %)	Satisfied with triage Scored 7–10 (valid %)	Given pain relief Answered yes (valid %)*	Wait time (WT) Mean WT (minutes)	Satisfied with wait Scored 7–10 (valid %)
Age group					
14-24 years	18 (66.7%)	21 (75%)	10 (45.5%)	161.5	15 (46.4%)
25-44 years	55 (87.3%)	61 (93.8%)	28 (82.4%)	110.4	22 (66.2%)
45-64 years	51 (92.7%)	53 (89.8%)	16 (50%)	75.4	17 (71.2%)
65 years and over	58 (92.1%)	62 (84.9%)	14 (34.1%)	112.5	20 (72.6%)
Gender					
Female	87 (83.7%)	99 (84.6%)	40 (58%)	140.0	69 (59%)
Male	92 (92%)	94 (90.4%)	27 (47.4%)	77.9	80 (76.9%)
Primary language					
English	166 (87.4%)	180 (87.4%)	64 (53.3%)	111.2	137 (66.5%)
Other than English	13 (86.7%)	14 (87.5%)	4 (57.1%)	92.4	12 (75%)
Country of birth					
Australia	115 (89.1%)	122 (89.1%)	43 (54.4%)	93.1	99 (72.3%)
Other than Australia	64 (84.2%)	72 (84.7%)	25 (52.1%)	138.6	50 (58.8%)
Culturally and linguistically diverse					
Yes	16 (76.2%)	24 (100%)	9 (47.4%)	111.8	16 (66.7%)
No	159 (89.3%)	165 (95.9%)	58 (32.8%)	111.0	129 (67.2%)
Person with a disability					
Yes	22 (88%)	26 (89.7%)	8 (53.3%)	101.7	19 (65.5%)
No	153 (87.9%)	163 (87.2%)	59 (54.6%)	112.6	126 (67.4%)
Person with a chronic condition					
Yes	45 (90%)	50 (87.7%)	16 (43.2%)	97.1	43 (75.4%)
No	130 (87.2%)	139 (87.4%)	51 (59.3%)	115.9	102 (64.2%)
Person with a mental health condition					
Yes	29 (87.9%)	30 (85.7%)	11 (61.1%)	95.0	24 (68.6%)
No	146 (88%)	159 (87.8%)	56 (53.3%)	114.3	121 (66.9%)

Note. *percentage excludes 'did not require pain relief' responses. The sample of LGBTIQA + gender diverse patients (n = 6) was too small for meaningful comparison and interpretation, thus not included.

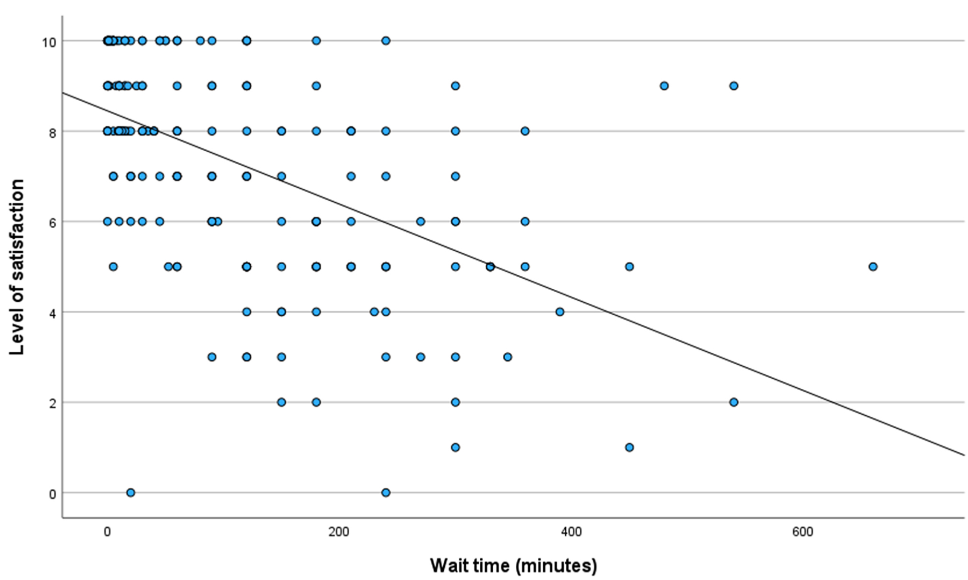

Fig. 1. Scatter plot displaying linear relationship between wait time and level of satisfaction with wait.

potential improvements at triage or in the waiting area (Table 4). *Systems and processes* highlighted respondents' desire for more staff and hospital beds and more efficient processes to reduce wait times. *Environment and facilities* reflected the need for better seating, more facilities and a nicer environment for patients waiting. *Communication and information* predominantly reflected respondents' expectations for

Table 4. Suggestions for improvement.

Category	Sub-category (codes)	Example comments
Systems and processes	Better access to care (2)	"less wait time"
	Less wait time (15)	"maybe a ticket system"
	More efficient processes (7)	"more people behind the counter"
	More staff, more hospital beds (16)	"very under resourced for the size of the region – more beds/doctors needed"
	Regularly check-in with patients (3)	
Environment and facilities	Access to food/drink (7)	"TV would be good – with hospital info"
	Better facilities (11)	"place to lay down important"
	Better layout (6)	"seating uncomfortable"
	Better seating (30)	"waiting room unclean - patients leaving rubbish around"
	Cleaner environment (6)	"it was very cold in there"
	More pleasant environment (10)	
Communication and information	Better communication with staff (12)	"If I were to be informed about a plan of waiting time then I would have felt more relaxed and less anxious and I would not mind waiting"
	Better understanding of processes & care options (5)	
	Information about wait time (26)	
Individual care needs	Considerations for individuals (11)	"can see others who can go through to urgent care"
	Initiation of care (1)	"thought my condition would have been seen quicker"
	Pain relief (10)	"pain relief after 8 hours - had to ask"
	Previous negative experience (4)	"in the past I felt forgotten"

wait time information and communication with staff. *Individual care needs* indicated the care needs of individuals, including pain relief, should be considered.

4. Discussion

This study described patients' experiences of triage and the waiting room in Western Australian EDs. Results indicated that whilst patients commonly presented to the ED with pain and anxiety, and many endured long waits, they were often highly satisfied with their experience of triage and the waiting area. Triage experience and interactions with the triage nurse were largely reported as positive, except for receiving inadequate information about wait times. Elements of waiting experience were variable. Reported wait times ranged from 0 to 11 hours, with satisfaction levels negatively correlated with wait time. Females, young adults and people born outside of Australia reported higher wait times than their counterparts. Only half of patients who desired pain relief reported they received it, least frequently for people over 65 years. Patients often acknowledged that staff were doing their best and suggested that better seating, information about wait time and better communication with staff could improve their experience. Results suggest a more person-centred approach could be adopted.

Satisfaction with triage staff was consistently reported as high. Aspects of interpersonal communication; being polite, listening, taking the patient seriously, asking about pain and instilling confidence were identified by over 90% of respondents, and suggest these skills were well developed in the triage nurses. This aligns with national data for patient experience 2023-4 which reported that 86.8%, 88.7, and 83.3% of patients indicated the ED nurses listened carefully, showed respect and spent enough time with them, respectively.[36] Areas for improvement were triage nurse introducing themselves and provision of wait time information. Patients' desire for wait time information has been reported by others.[16] The CQI A&E previously identified similarly higher scores for 'attitude of health care professionals' and 'professionalism of received care' and lower experience scores for the 'information before treatment' domain, which included information on the rapidity and order of treatment.[28] In their qualitative meta-synthesis, Graham et al.[12] highlighted patients' desire for wait times and suggested that providing wait time information could help mitigate negative effects of waiting, such as anxiety, and improve their experience.

Unsurprisingly, wait time was the key element that impacted patient experience at triage. Like other studies, this study identified that irrespective of the overall triage experience length of wait time was negatively associated with patient satisfaction.[37–40] The patient-reported wait time was longer (median 60 minutes and longest 660 minutes), compared with 2024 Australian national wait time statistics of 18 minutes (median) and 117 minutes (90th percentile).[32] An integrative review synthesised wait times from three studies from 2011–2021 and reported a mean wait time of 30 minutes.[16] In the current study, wait times were longer in metropolitan than rural areas, which aligns with the high number of patients presenting to these sites. Prolonged wait times are a reported consequence of ED overcrowding, access block, and staff shortages that continue to be an issue in Australian EDs.[41–43] However, many patients seemed to accept the long wait time, acknowledging system-related barriers and the need for more staff and beds. Janerka et al[16] suggested delivery of person-centred care while waiting.

Patients highlighted the need for better communication, information, facilities and comfort whilst they were waiting for medical assessment. Only half reported feeling comfortable in the waiting area, and many suggested better seating and access to food, drinks and analgesia. Pain relief was provided to only half of those patients who desired it (and inconsistency across age groups), and few patients reported being updated about their wait time, despite the apparent availability of staff noted by most. Patients' desire for information about ED processes and wait times and the link with satisfaction has been reported since the 1990s and continues to be inadequately provided.[16,44,45] Similarly, delayed pain relief is an ongoing issue suffered by ED patients, who are often required to wait for medical assessment before being prescribed analgesia.[12,46] In particular, older adults may be less likely to receive pain relief than other adults,[47] as was reported in the current study.

Patients in this study identified areas for improvement and provided suggestions that constitute person-centred care at triage and while waiting to be assessed by a doctor. Firstly, providing information about wait time, in person at triage or displayed in the waiting area, appears to be a simple action that could improve patient experience.[48–50] Local policies, logistical issues, and possible reluctance by triage nurses warrant further investigation to support effective implementation. Secondly, if patients are expected to wait prolonged times in uncomfortable waiting areas, then EDs should invest in facilities (such as comfortable seating) and staff to support the provision of meaningful care in these areas. Designated waiting room staff is recommended, such as volunteers or waiting room nurse; a nurse allocated specifically to communicate with patients and their families and provide person-centred care in the waiting room.[51] Finally, the provision of and timely pain relief can be supported by nurse-led analgesia protocols at triage or by waiting room staff.[52] Although such interventions have demonstrated positive outcomes, their implementation has often been conducted in isolation, with limited information on long-term sustainability. Additionally, the variability in triage and waiting room designs, processes and models of care can challenge effort to standardise these interventions.[53] To enhance patient experience, there is need for a standardised approach using practical guidelines for implementing targeted person-centred interventions in ED triage settings and evaluating effectiveness.

4.1. Limitations

This study has some limitations. Many survey questions used a three-point response scale ('yes', 'no' and 'somewhat'), which may limit the sensitivity of the instrument and contribute to little variance in the data. This could potentially lead to positively skewed results or a ceiling effect.[54] However, responses to the open-ended questions including in this survey largely supported the quantitative findings. Patients who were too unwell or who left the ED prior to medical assessment were not able to be recruited, and so perspectives of these patients were not captured. Previous studies have reported long wait times as a common reason for patients to leave the waiting room before care.[16] Another possible limitation, as noted by a member of the project reference group, is that of recall bias. Patients were recruited after being brought into the main department for assessment and treatment, so may have felt more positive about their experience when completing the survey than while in the waiting room. Data collection was retrospective so the patient's whole ED experience was considered. Future research could consider point in time or observational research of patient experience in the waiting room.

5. Conclusion

This study highlights that patients are often highly satisfied with their triage encounter, yet uncertainty about or long wait time continues to be a dominant issue influencing their ED experience. Reducing wait times is a priority for EDs however, there should also be attention on providing person-centred care for patients who are waiting. Better seating and facilities, provision of wait time information and updates, pain relief and individualised care is recommended. The findings from this study contribute to a broader research initiative aimed at developing a person-centred approach to ED triage and the waiting room.

Conflict of interest

The authors have no competing interests to declare.

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Appendix I: Patient Survey

This survey is to find out about your experience (what happened and how you felt) when you first came to the Emergency Department (ED) today, at **triage** and in the **waiting area**.

Triage is when the nurse at the window asked questions to find out how unwell you are. This nurse is the **triage nurse**. You might have been triaged on an ambulance trolley.

The **waiting area** is the waiting room or ambulance ramp where you were waiting <u>before</u> you were brought into the main department to see a doctor/nurse practitioner. You may have been brought straight in and not waited.

The survey will take about 15 minutes to complete. The researcher or a carer/family member can help if needed. A translator can be arranged if needed.

You must be over 14 years old and spoke with the triage nurse in person when you first arrived at ED.

People under 18 years old require parent or guardian consent to participate in this research.

For young people aged 14–17 years old:

[ ] Parent/guardian has given consent to participate in this research.

[ ] Requisite capacity and maturity to understand the nature and demands of the research, without vulnerability, has been deemed by the Researcher.

Section 1: Information about you

This section collects information about people who completes the survey. This is so when we put all the survey results together, we can see if a range of different people are represented.

1. How old are you?

- ☐ 14–17 years
- ☐ 18–24 years
- ☐ 25–34 years
- ☐ 35–44 years
- ☐ 45–54 years
- ☐ 55–64 years
- ☐ 65–74 years
- ☐ 75 and older
- ☐ I'd prefer not to say

2. Are you?

- ☐ Female
- ☐ Male
- ☐ Non-binary
- ☐ Other (please comment below)
- ☐ I'd prefer not to say

Comment: ____

3. What language do you mostly speak at home? ____________________

4. What is your country of birth? ______________________________

5. Do you identify with any of the below? (can select multiple answers)

☐ Culturally and linguistically diverse
☐ Person with disability
☐ Person with chronic condition/s (such as diabetes, heart disease, etc.)
☐ Person experiencing or has experienced mental health problem/s
☐ LGBTIQA+
☐ I don't identify with any of these
☐ I'd prefer not to say

6. Is there anything else you would like us to know about you that might affect your triage experience?

____

Section 2: Before triage

The following questions relate to your experience prior to and when you first arrived at the ED, before you were triaged.

7. Were you told to come to ED by a doctor/GP or other health professional?

☐ Yes
☐ No
☐ Unsure

8. How serious, according to you, was the health problem for which you visited the ED?

☐ Not serious
☐ Somewhat serious
☐ Serious
☐ Very serious

9. On a scale of 0 to 10, how anxious or fearful were you about your health problem for which you visited the ED? (circle a number)

0 = calm, at ease, 10 = extremely anxious/fearful.

0 1 2 3 4 5 6 7 8 9 10

10. On a scale of 0 to 10, how much pain did you have when you arrived at the ED? (circle a number)

0 = no pain, 10 = most terrible pain imaginable.

0 1 2 3 4 5 6 7 8 9 10

Section 3: During triage

The following questions relate to your experience during triage. This is when you saw the nurse at the window (usually) who asked what brought you into ED today and they entered notes about your problem into the computer system.

11. Triage is when the nurse assesses how unwell you are and how quickly you need to see a doctor. Did you know this process prior to coming to the ED today?

☐ Yes
☐ No
☐ Somewhat

12. How long did you have to wait before you first spoke to the triage nurse? ____________________

13. Did the triage nurse introduce themself?

☐ Yes
☐ No
☐ Unsure

14. Was the triage nurse polite to you?

☐ Yes
☐ No
☐ Somewhat

15. Did the triage nurse take you seriously?

☐ Yes
☐ No
☐ Somewhat

16. Did the triage nurse listen carefully to you?

☐ Yes
☐ No
☐ Somewhat

17. Did the triage nurse have enough time for you?

☐ Yes
☐ No
☐ Somewhat

18. Did the triage nurse ask you if you were in pain?

☐ Yes
☐ No
☐ Somewhat

19. Did you have confidence in the triage nurse?

☐ Yes
☐ No
☐ Somewhat

20. Were you told how long you would have to wait to be seen by a doctor/nurse practitioner?

☐ Yes
☐ No

☐ Somewhat

21. Were you told the reason why you had to wait to be seen by a doctor/nurse practitioner?

☐ Yes
☐ No
☐ Somewhat

22. Overall, did you get the care you expected from the triage nurse?

☐ Yes
☐ No
☐ Somewhat

23. On a scale of 0 to 10, how satisfied were you with your experience of triage? (circle a number)

0 = highly dissatisfied, 10 = highly satisfied

0 1 2 3 4 5 6 7 8 9 10

Section 4: After triage (Waiting Area)

The following questions relate to your experience in the waiting area (waiting room or ambulance ramp), after triage and before you were taken into the department to be seen by a doctor/nurse practitioner.

24. Where did you wait?

☐ Waiting room
☐ Ambulance ramp
☐ I went straight in
☐ Other ______________________________

25. Were you given pain relief at triage or in the waiting area?

☐ Yes
☐ No
☐ Not required

26. Did you have a family member, friend or carer with you at triage or in the waiting area?

☐ Yes
☐ No

27. If you needed assistance, were you able to get a member of staff to help you?

☐ Yes
☐ No
☐ Somewhat

28. How long did you have to wait in the waiting area before being taken into the main department?

____

29. Did you have to wait longer because more serious patients were treated first?

☐ Yes
☐ No
☐ Unsure

30. Were you updated about your wait time?

☐ Yes
☐ No
☐ Somewhat

31. Did you ever feel worried that staff in the emergency department had forgotten about you?

☐ Yes
☐ No
☐ Somewhat

32. Was the waiting area comfortable?

☐ Yes
☐ No
☐ Somewhat

33. Did you feel safe in the waiting area?

☐ Yes
☐ No
☐ Somewhat

34. On a scale of 0 to 10, how satisfied were you with your experience of the waiting area. (circle a number)

0 = highly dissatisfied, 10 = highly satisfied

0 1 2 3 4 5 6 7 8 9 10

35. Was there anything about your time at triage or in the waiting area that you think could have been improved?

____

____

Appendix II: Survey tool validity testing

	Priori criterion for 7 raters (Imle & Atwood, 1988)	Items that met criterion	Panel agreement
Clarity	70% agreement for each item and instructions and 85% overall	3/3 instructions 29/29 items	96% overall
Apparent internal consistency	70% agreement for each item and 85% overall	3/3 instructions 29/29 items	100% overall
Content validity, each subscale:			100% overall
Prior to triage	86% agreement for fit and 86% for uniqueness of each subscale set		100% fit 100% uniqueness
During triage			100% fit 100% uniqueness
After triage			100% fit 100% uniqueness

Appendix III: Assessment of Child's Vulnerability and Capacity to Participate in Research

Vulnerability Considerations (developing the research)

Perceived or real vulnerability	Remediation
Poorly designed study	Study designed in accordance with the National Statement on Ethical Conduct in Human Research, particularly Chapter 4.2: Children and Young People.
Inadequate or biased consent discussion	Consent will be explained verbally with the child and parent/guardian and presented in written form. Researcher will reinforce that the child does not have to participate in the research if they do not wish.
Material presented in a way potential participant cannot understand	The survey and information sheet will be worded at a readability level of 12 years old.
Pressure by investigator to quickly decide	The child and parent/guardian will be given time to discuss their decision in private. The Researcher will be friendly and understanding.
Stressful situation	Child is not suitable to participate if they are visibly or self-claim to be distressed or in severe pain.
Unable to read	Not excluded from invitation to participate, parent/guardian or Researcher can read aloud.
Power differential	Researcher will use age-appropriate dialogue. Researcher will not be involved in the provision of care at the health site. Child will be reassured that the survey will not be shown to the ED staff and will not impact ED care in any way.

Note. Adapted from Biros M. Capacity, vunerability, and informed consent for research. Journal of Law, Medicine & Ethics. 2018;46(1):72–78; and Havenga Y, Temane MA. Consent by children: considerations when assessing maturity and mental capacity. South African Family Practice. 2016;58(sup1):S43–S46.

Evaluation to Consent Questionnaire:

Question	Acceptable Answers
1. What is a potential risk?	I might feel uncomfortable re-telling my triage experience
2. What is expected from you, the participant?	Answer survey questions Talk about my triage experience in an interview
3. What if you don't want to continue?	I can ask to stop or mention that I don't feel comfortable, don't want to answer any more questions, or have changed my mind about being a participant.
4. What if you experience discomfort?	Let the Researcher or my family member/carer (if present) know that I am feeling uncomfortable. The Researcher might notice that I look uncomfortable and ask me if I want to stop or continue.
5. How is it decided who gets to participate in the study	The Researcher is asking anyone 14 or over in ED if they want to participate.

Note. Adapted from Resnick B, Gruber-Baldini AL, Pretzer-Aboff I, Galik E, Buie VC, Russ K, et al. Reliability and Validity of the Evaluation to Sign Consent Measure. The Gerontologist. 2007;47(1):69–77.